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As active brokers and capital facilitators, we constantly engage with investors, operators, and lenders to stay abreast of the Seniors Housing and Long-Term Care market. This brief report highlights key insights gathered from recent conversations with industry stakeholders of all types.

- Home Health, Hospice, and other ancillary healthcare companies have become more aggressive purchasers of senior communities. It legitimizes these groups to have ownership of real estate in their markets and they can extract more out of cost departments than a traditional seniors housing investor could. This is most applicable for smaller purchases (<$5MM) as the capital required for larger purchases is hard to raise for an ancillary service company.
- As newly built Assisted Living and Memory Care communities remain underoccupied, some regional chains are using low entry rates to fill units, with the intention to reassess the resident 3-6 months after moving in. More established communities are facing challenges to educate the market of these risks, as families' online searches allow them to quickly commoditize communities and filter based on pricing. This is putting pressure on established providers to conduct similar tactics, so they can at least be granted the chance to tour a prospective resident.
- With development costs high, investors are looking towards conversion projects in the Assisted Living and Memory Care space. The most relevant metric to understanding if a property can be converted is analyzing its billable square footage. Ideally this is at least 75% of the total square footage in a building. Too much common area and dead space can make a community inefficient as a senior property unless the focus is an amenities-rich, Class A product.
- Unique state-specific contracts are being created by Skilled Nursing facilities for resident care. Two recent examples are a provider starting an opioid nurse aid training program and a contract with a state prison system to care for residents. Often, these facilities contact their legislatures themselves to create these contracts and they may act as pilot programs in their respective states.
- Private money and non-bank lenders are facing balance sheet risk as their maturing seniors housing loans aren't being paid off. While still issuing terms on new deals, many are facing execution uncertainty, and their funding sources need to be scrutinized before engaging. Lenders with warehouse lines typically have stronger execution ability than those with CLO's, as their funding liquidity is more guaranteed.
- Opportunistic Buyers' acquisition filters are becoming narrower as there is no shortage of distressed inventory on the market. Some super-niche examples are groups that seek memory care facilities that are within a certain mile radius to a specific chain operator or targeting communities with an outdated website design.
- Many hospitals have stopped allowing SNF directors and vendors to visit their case managers. This has prompted SNF's to expand their search for hospital referral sources outside of their respective counties.
- Continued delay in Employee Retention Credit funding for providers has created a market of financing companies who purchase ERC receivables. These funds view this as investing in a federal government receivable and are buying them at 75-80% of their face value.
- SNF's are experiencing an increase in hospital referrals by opening specialized subacute units. The most common focus is on respiratory care.
- The private pay Assisted Living market is becoming increasingly aware of state Medicaid waiver residents that are co-living in their community. This contrast becomes especially highlighted if communities take in residents that require care beyond basic ADL needs and/or have behavioral issues. It's resulted in providers becoming more selective with Medicaid Waiver residents. In California for instance, a provider may now review twenty Medicaid Waiver applicants before finding the right fit. Naturally, this has increased the costs of admissions for these residents as fulltime staff is required to vet applicants and enroll in the program.

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